PRINTABLE KIT · 11 PAGES

FOR THE WEEK THAT DECIDES WHETHER THEY GO BACK

After the Hospital: The First 7 Days Home

What to get before you leave the hospital, how to sort the new medications from the old, what to watch for each day, and who's covering which shift.

WHAT'S INSIDE

- Before you leave the hospital
- Medication changes sheet
- The 7-day plan
- When to call, when to go
- Follow-ups & services tracker
- Daily vitals & symptom log
- Setting up the house
- The help roster
- Questions for the follow-up visit
- Call log

DISCHARGE DAY · IN THE HOSPITAL ROOM

Before you leave the hospital

Discharge happens fast, usually when you're both tired. Most of what goes wrong in the first week traces back to something that wasn't asked here. Check these off before the wheelchair arrives.

Papers in hand

- [ ] Discharge summary, printed
  Diagnosis, what was done, what's next, in writing.
- [ ] The new medication list, printed
  Copy it onto page 3 with the nurse while they're in the room.
- [ ] Written care instructions
  Wound care, diet, activity limits, bathing, lifting.
- [ ] Follow-up appointments, with dates
  If they say "call to schedule," ask them to book it now.
- [ ] Prescriptions sent to the pharmacy, confirmed
  Ask which ones are new and which replace something.
- [ ] The after-hours number for the team that treated them

Questions for the discharge nurse

- [ ] What should I watch for, and what exactly do I do if I see it?
- [ ] Which symptoms mean call you, and which mean call 911?
- [ ] What was stopped, changed, or added on the medication list, and why?
- [ ] Can they shower? Climb stairs? Drive? Be alone? For how long?
- [ ] What equipment do they need at home, and who is delivering it?
- [ ] Is home health, physical therapy, or a visiting nurse ordered? Who calls whom?
- [ ] What's the diet, in plain words?
- [ ] When is the next appointment, and what should we bring?

Before you drive away

- [ ] Pharmacy has the prescriptions and they're ready, or you know when
- [ ] Equipment (walker, oxygen, commode, bed) is home or arriving today
- [ ] Someone is staying the first night
- [ ] Fridge has food that matches the diet
- [ ] Pathway from car to bed is clear
- [ ] Page 4 of this kit is filled in from the discharge papers

This kit organizes the instructions your care team gives you. It does not replace them. When this kit and your discharge papers disagree, the discharge papers win, and the after-hours number is the tiebreaker.

DISCHARGE DAY · FILL IN WITH THE NURSE, CONFIRM WITH THE PHARMACIST

Medication changes sheet

Readmissions often start with two lists in the house: the old bottles in the cabinet and the new ones from the pharmacy. This page is the one list. Then clear the old bottles out.

MEDICATION & STRENGTH	STATUS	DOSE & WHEN	WHAT IT'S FOR	NOTES (REPLACES WHAT? WITH FOOD? STOP DATE?)
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
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	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			
	☐ New ☐ Changed ☐ Same ☐ Stopped			

Three questions for the pharmacist

- ☐ Is anything on this list a duplicate of something they already take under another name?
- ☐ Does anything here interact with their old medications or supplements?
- ☐ Which of these can be stopped on a date, and

Old bottles removed from the house (name, date)

FILL IN FROM THE DISCHARGE PAPERS · POST IT NEXT TO THE FRIDGE SHEET

When to call, when to go

Your discharge papers list the warning signs for this condition. Copy them into the boxes below so nobody has to find the paperwork at 2 a.m. The general signs printed here apply to almost every discharge.

Call 911 now

Chest pain or pressure · trouble breathing · face drooping, arm weakness, slurred speech · sudden severe confusion or can't wake them · heavy bleeding that won't stop · a fall with a head strike, or they can't get up · fever with shaking chills after surgery

Condition-specific, from the discharge papers:

Call the care team today (use the after-hours number if needed)

Fever · redness, swelling, warmth, or drainage at an incision or wound · new or worse pain not controlled by what was prescribed · not keeping food or fluids down · no urine for many hours, or burning · new swelling in a leg · new confusion, dizziness, or a near-fall · a missed or doubled medication you're unsure about

Condition-specific, from the discharge papers:

Watch and note (bring to the follow-up)

Appetite, sleep, energy, mood, bathroom changes, mild soreness that's improving. Write it on the daily log; it's what the doctor will ask about.

WHO	NUMBER	WHEN TO USE IT
Care team after-hours line		Nights, weekends, anything on the yellow list
Primary care doctor		Follow-up, refills, questions in office hours
Pharmacy		Medication questions, missed doses
Home health / visiting nurse		Wound checks, vitals, therapy

THE WEEK

The 7-day plan

A default rhythm for the week, to adjust to your discharge instructions. Check boxes as you go. The daily log on page 7 holds the numbers.

Day 1
Home day

- [ ] Medication changes sheet done, old bottles out
- [ ] Pill organizer filled for the week
- [ ] "When to call" page filled and posted
- [ ] Fluids and a real meal, per the diet
- [ ] Someone stays overnight
- [ ] Confirm pharmacy pickup and equipment

Day 2
Settle

- [ ] First full day on the new medication schedule
- [ ] Call to confirm every follow-up on page 6
- [ ] Wound / incision check per instructions
- [ ] Short walk or exercises, if allowed
- [ ] Daily log started: vitals, pain, appetite
- [ ] Help roster filled for the week

Day 3
Watch

- [ ] Compare today's log to yesterday's; note anything worse
- [ ] Bathroom and fluids on track?
- [ ] Pain controlled with what was prescribed?
- [ ] Home health / nurse visit, if ordered
- [ ] Refill anything running low

Day 4
Midweek

- [ ] Caregiver check: did you sleep? Who relieves you?
- [ ] Activity up a notch only if the instructions allow
- [ ] Write questions for the follow-up (page 10)
- [ ] Fridge: still matches the diet?

Day 5
Routine

- [ ] Medications: any side effects to report?
- [ ] Wound / incision check
- [ ] Mood and sleep: note changes
- [ ] Call the office with yellow-list items before the weekend

Day 6
Prepare

- [ ] Gather for the follow-up: this kit, med list, daily log, discharge papers
- [ ] Confirm ride and time
- [ ] Pill organizer refilled for next week

Day 7
Review

- [ ] Read the week's log together: better, same, or worse?
- [ ] Decide what help continues and what can stop
- [ ] Follow-up visit (often within 7 to 14 days)
- [ ] Update the fridge sheet and medication list

DAY 1 TO 2

Follow-ups & services tracker

Everything that was "ordered" or "arranged" at discharge, with a date and a phone number. If a line has no date by day 2, call and get one.

WHAT	DATE & TIME	WHO / WHERE	PHONE	CONFIRMED? RIDE? NOTES
Primary care follow-up Usually within 7 to 14 days				
Specialist / surgeon follow-up				
Home health / visiting nurse First visit date				
Physical / occupational therapy				
Equipment delivery Walker, bed, oxygen, commode				
Lab work or imaging				
Pharmacy pickup / delivery				
Insurance / case manager call				

If something that was "ordered" hasn't called by day 3

Call the hospital's discharge planner or case manager (the number is usually on the discharge summary). Orders get lost between the hospital and the agency more often than anyone admits. Write the call on the log on page 11.

EVERY DAY THIS WEEK · BRING TO THE FOLLOW-UP

Daily vitals & symptom log

Only track what the care team asked for, plus the basics. A week of numbers side by side shows a trend that no single day does.

Week starting: _____ Target ranges from the care team (if given): BP _____ Weight _____ Other _____

	DAY 1	DAY 2	DAY 3	DAY 4	DAY 5	DAY 6	DAY 7
Temperature							
Blood pressure / pulse							
Weight							
Oxygen / breathing							
Pain 0–10							
Ate & drank							
Bathroom							
Sleep							
Mood & alertness							
Wound / incision							
Activity / exercises							
Meds all given?							

Anything new, different, or reported to the care team (date it)

BEFORE THEY COME HOME, OR DAY 1

Setting up the house

Recovery happens in about four places: bed, bathroom, chair, kitchen. Make the path between them short, lit, and clear, and put what they need within arm's reach of each.

Bed

- [ ] On the main floor if stairs are limited
- [ ] Height lets feet touch the floor
- [ ] Lamp, phone, water, glasses within reach
- [ ] Night light to the bathroom
- [ ] Extra pillows for positioning, if instructed
- [ ] Commode or urinal nearby if the bathroom is far

Bathroom

- [ ] Grab bars or a sturdy hold at toilet and shower
- [ ] Shower chair and handheld sprayer if bathing is allowed
- [ ] Non-slip mat; rugs removed
- [ ] Raised toilet seat if standing up is hard

Chair & living space

- [ ] A firm chair with arms, not the low sofa
- [ ] Walker or cane parked where they sit
- [ ] Remote, phone, water, tissues, medications list at hand
- [ ] Cords, rugs, pet beds out of the path
- [ ] Chair to bathroom and chair to kitchen both clear

Kitchen

- [ ] Meals for the week matching the diet, portioned
- [ ] Fluids they'll actually drink
- [ ] Everyday items at counter height
- [ ] Pill organizer on the counter, filled

Equipment

- [ ] Walker / cane adjusted to their height
- [ ] Oxygen set up and explained, tubing routed safely
- [ ] Hospital bed, if ordered, placed and tested
- [ ] Thermometer, BP cuff, scale if the team asked for readings
- [ ] Wound supplies in one bin

Paper

- [ ] "When to call" page posted
- [ ] Medication list on the fridge
- [ ] Discharge papers in one folder
- [ ] This kit on the counter

STILL NEEDED	WHO	BY	DONE

DAY 1 · FILL IN WITH WHOEVER IS HELPING

The help roster

One person can't do seven nights. Write names in boxes, not "we'll figure it out." Friends who ask "what can I do?" get a box.

	DAY 1	DAY 2	DAY 3	DAY 4	DAY 5	DAY 6	DAY 7
Morning Meds, breakfast, wash							
Midday Lunch, exercises, check-in							
Evening Dinner, meds, wound check							
Overnight							
Rides & errands Pharmacy, appointments							
Meals dropped off							

Helpers & how to reach them

NAME	PHONE	GOOD FOR (NIGHTS, RIDES, MEALS, SITTING, DOG)

For the primary caregiver

Your box on this grid matters as much as theirs. If your name is in every row, that's the first problem to fix, before day 3, while people are still offering.

DAY 4 TO 6 · BRING TO THE FIRST VISIT

Questions for the follow-up visit

Check the ones that apply, add your own from the week's log, and write the answers during the visit. Bring this kit, the medication sheet, and the daily log.

Appointment with: _____ Date: _____

About recovery

- [ ] Is this where you'd expect them to be a week out?
- [ ] Here's the daily log. Does anything in it concern you?
- [ ] What should improve next, and by when?
- [ ] What activity is allowed now: stairs, driving, lifting, bathing, being alone?
- [ ] What would make you want to see them sooner?

About medications

- [ ] Here's the changes sheet. Is this list right?
- [ ] Which of the new medications stop, and when?
- [ ] We noticed _____ since starting _____. Related?
- [ ] Can the old prescriptions be cancelled at the pharmacy?

About services

- [ ] Should home health or therapy continue, change, or stop?
- [ ] Is any equipment no longer needed, or newly needed?
- [ ] Who coordinates between you and the specialist?
- [ ] Which release lets you talk with me directly?
- [ ] When is the next visit, and with whom?

Our own questions

Answers & notes from the visit

ONGOING

Call log

Every call with the care team, the pharmacy, the agency, insurance, or the equipment company. Date, who, what was said, what happens next. Ask for a reference number on insurance calls.

DATE	WHO I SPOKE WITH	ABOUT	WHAT WAS SAID / DECIDED	FOLLOW-UP & REFERENCE #

After the first week

If care continues past this week, the companion kit *The First 30 Days* carries on from here: the fridge sheet, the full medication list and weekly log, the documents checklist, and a weekly check-in for the caregiver.

This kit is an organizer for the instructions your care team gives you. It is not medical advice. The warning signs printed here are general; the ones in your discharge papers are specific to this hospital stay and take priority. When in doubt, call the number on page 4.